NCCP 00253 · VERSION 6

Tamoxifen · daily hormone therapy

Take tamoxifen once daily continuously. There is no planned week off. The prescriber confirms the duration for the treatment setting.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Tamoxifen: material risks

☐ Hot flushes, sweating and sleep disturbance

☐ Blood clots in the leg or lung; stroke

☐ Visual disturbance / cataracts

☐ QT rhythm risk in susceptible patients

☐ Vaginal / sexual symptoms and menstrual changes

☐ Endometrial changes and cancer risk

☐ Liver effects and uncommon serious skin reactions

History of significant thromboembolic disease is an NCCP exclusion. Severe hepatic impairment is not recommended. Interactions, including potent CYP2D6 inhibitors, need medication review. Tamoxifen is not contraception. Discuss non-hormonal contraception during treatment and for nine months after stopping, where relevant; absent periods do not prove infertility.

Individual benefit, alternatives and concerns

Agreed checks and next review

FBC, renal and liver profile before treatment. Check clot history, medicines and pregnancy possibility. ECG and electrolytes are added when cardiac / QT risk makes them clinically indicated. Liver tests, INR, ophthalmology and ECG are repeated when clinically indicated. NCCP recommends an annual gynaecological examination; new abnormal vaginal bleeding needs prompt assessment.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.2 · Sources checked 2026-10-18
Source reconciled; independent clinical, pharmacy and patient/PPI review pending